

UHB URGENT CANCER REFERRAL FORM FOR SUSPECTED UROLOGICAL CANCERS

INDICATES **MANDATORY FIELDS** – For Bladder and Prostate cancer please tick UTI has been excluded or the referral MAY NOT BE ACCEPTED

PATIENT DETAILS		GP DETAILS
Name:	Title Given Name Surname	Practice Name: Organisation Name Usual GP Name: Usual GP Full Name
Address:	Home Full Address (stacked)	Practice Address: Organisation Full Address (stacked) Practice Code: Organisation National Practice Code Phone No: Organisation Telephone Number
NHS Number:	NHS Number	Practice Email: Organisation E-mail Address
Hospital number:	Hospital Number	Name of referrer: Free Text Prompt
Date of Birth:	Date of Birth	Date of referral: Short date letter merged
Interpreter required	☐ Yes Language: Main Language	Referrer contact details (if different from above/GP Practice)
Sign Language required:	☐ Yes Language: Main Language	
Contact Details (next 48 hrs):	Mobile: Patient Home Telephone Home: Patient Mobile Telephone Work: Patient Work Telephone Email: Patient E-mail Address	
GP Declaration – Please confirm and tick	☐ I have informed the patient they have symptoms which may be caused by cancer, that they are being referred urgently, and the nature of the tests likely to take place. ☐ I have provided the patient with an Urgent Referral Patient Information Leaflet. ☐ The patient has confirmed they are available to attend within the next 2 weeks and is aware they might go Straight To Test (prior to any other hospital contact). ☐ The patient is aware that they will be offered the first available appointment at any one of our hospitals (Queen Elizabeth, Heartlands, Solihull or Good Hope).	

☐ The patient is aware that they might be contacted for updates from UHB by any means (telephone call, letter, text).

REASON FOR REFERRAL

(All patients)

WHO Performance Status:	☐ 0 Fully active ☐ 1 Able to carry out light work ☐ 2 Up and about greater than 50% of waking time ☐ 3 Confined to bed/chair for greater than 50% ☐ 4 Confined to bed/chair 100%	
Accessibility Needs:	☐ Wheelchair access ☐ Deaf ☐ Registered blind ☐ Learning Disability ☐ Other disability needing consideration ☐ Accompanied by carer	
RISKS:	☐ Vulnerable Adult (detail if any recording within last 3 years*) *Detail ☐ No Capacity to Consent ☐ Any other known risk:	
Results (within last 3 months)	☐ Creatinine Single Code Entry: Serum creatinine level ☐ eGFR Single Code Entry: GFR (glomerular filtration rate) calculated by abbreviated Modification of Diet in Renal Disease Study Group calculation OR ☐ Bloods have been requested – Results will be available by the time the patient is seen	
Diabetes:	YES ☐ NO ☐	Details :
Anticoagulation:	YES ☐ NO ☐	Details :
All suspected cancer sites	Abnormal radiological findings suggestive of urological malignancy. Please select the appropriate sub-site and attach the relevant report. Abnormalities in radiological investigations suggestive of malignancy override the typical symptoms in the sections below and qualify for this referral pathway.	Prostate ☐ Bladder ☐ Kidneys ☐ Ureter ☐ Testicles ☐ Penis ☐
Bladder/Renal (If the UTI excluded box has not been ticked the form will be returned)	UTI has been excluded by Dipstick ☐ OR MSU ☐ Date: (If MSU abnormal please treat UTI and refer to FAQ document)	
	Unexplained* visible haematuria without UTI ≥45 years.	☐

Prostate		OR >7.5 & symptomatic (bone pain / weight loss)	the guidance is to repeat in 6 months and to refer via 2ww if referral criteria met or if PSA has doubled and the patient has a WHO performance status of 0 or 1. If patients do not fit the outlined criteria but concerns remain, seek appropriate support via "advice and guidance".	
	DRE* - (Mandatory only if PSA is normal/borderline)	Benign (soft/smooth) ☐	Abnormal (hard/irregular/nodular/asymmetric) ☐	
	MRI Checklist	Does the patient have:	Y	N
		Pacemaker	YES ☐	NO ☐
		Cranial aneurysm clip / stent / coil	YES ☐	NO ☐
		Orbital / facial metallic implants	YES ☐	NO ☐
		Hip replacement	YES ☐	NO ☐
Implanted device/prostheses		YES ☐	NO ☐	
	Any recent surgery (<3 months)	YES ☐	NO ☐	
Family history of Prostate Cancer	YES ☐ NO ☐ UNKNOWN ☐		If yes, details :	

In patients compromised by co-morbidities or with a below 10 year life expectancy, a discussion with a urologist may be more appropriate. Please consider whether the patient is fit for radical treatment or whether an urgent referral might be more apt.

Relevant additional history/comments (including medications, allergies, major medical history)

Problems

Medication

Allergies